Last update: 2017/2018

Psycho-oncology for gynecologic malignancies

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Definition

Psycho-oncology is the study of the relationships of cancer and the psyche. It is a subarea of oncology in which experts from various areas (psychology, social work, psychiatry, oncology, gynecology, nursing, art and music therapy, nutrition counseling, and spiritual care) jointly address the emotional reactions of the patient and her family. Psycho-oncology focuses on identifying psychosocial factors influencing morbidity and mortality and on supporting the patients and their families in coping.

Definitions of psycho-oncological counseling and psycho-oncological therapy

The distinction between psycho-oncological counseling and psycho-oncological therapy is clinically relevant and was defined as follows in 2007 by the Swiss Society of Psycho-Oncology (SGPO) in its guideline on psycho-oncology (Leitlinien Psychoonkologie):
Psycho-oncological counseling includes counseling, psychoeducation, and ongoing support for cancer patients and their families and can be carried out by qualified nurses, social workers, psychologists, physicians, or other qualified personnel. Psycho-oncological therapy additionally includes, in particular, psychological diagnostics, crisis intervention, and psycho-oncological psychotherapy of patients and their families with significant mental impairment or comorbid mental health issues and must be carried out by psycho-oncologists with psychotherapeutic training.

Swiss Society of Psycho-Oncology

In the beginnings of psycho-oncology, questions were raised about a cancer personality and the influence of critical life events on pathogenesis, which CANNOT be maintained from today's perspective. However, patients often wonder why they have the disease and develop their own, subjective disease theory.

Most frequent stressors in gynecologic malignancies

Carcinoma is associated with general and locality-specific as well as treatment-related stressors.

General Stressors

- Vital threat and confrontation with the finite nature of life
- Limited lifetime in palliative situations
- Distress caused by medical procedures (see below)
- Situational adaptation requirements: Changes in treatment staff, Feeling of helplessness, Appointment stress
- Decision-making conflicts
- Social changes: workflow, changing role in partnership and family, friends
- Worries about family members
- Difficulty coping with everyday life
- Fear of follow-up visits and relapse

Treatment-Related Stressors

The patient is confronted with the following treatment-related changes, with which she can cope with varying success, on the basis of her previous development and her general distress level on the one hand, and her resources on the other:

Surgical Interventions

- Limited privacy during inpatient stay
- Changes in body image, organ loss → subjective; possibly experienced as a loss of femininity and violation of gender identity.
- Pain
- Limited motility due to surgery and lymphedema
- Genital surgery: possible loss of reproductive capacity, hormonal changes

Chemotherapy

- Alopecia --> looking sick, loss of femininity
- Malaise and weakness --> greatly reduced performance
- Possible hormonal changes --> climacteric symptoms, mood swings
- Loss of reproductive capacity
- Tiredness up to chronic fatigue

- Gastrointestinal symptoms (e.g., vomiting, diarrhea, constipation)
- Cognitive impairment --> concentration difficulties, memory decline (as a result of chemotherapy; colloquially called “chemo brain”)
- Mucous membrane changes

Radiation

- Burns
- Feelings of helplessness, especially in case of intracavitary radiation
- Tiredness up to chronic fatigue --> general exhaustion, excessive tiredness with excessive need for sleep, listlessness, feeling to no longer satisfy usual requirements, weakness
- Lymphedema

Anti-hormonal therapy

- Climacteric symptoms
- Loss of reproductive capacity
- Lack of lubrication and sexual problems

Psychological reactions

General Reactions

Carcinoma is considered to be a critical life event triggering a stress reaction in most cases. This reaction is seen as a normal reaction to an exceptional and threatening event.

The most common emotional reaction prior to diagnosis and/or prior to the availability of all medical findings is anticipatory anxiety, which can manifest itself more strongly than the reaction during the discussion of all findings. Consequently, many patients experience this waiting period as one of the most difficult phases.

The most common negative cognitive emotional reactions to diagnosis and during the course of the disease are:

- Shock
- Disbelief up to denial
- Fear, panic, and fear of the future
- Despair, hopelessness, and depressive reaction
- Numbness, feeling of emptiness
- Anger, aggression, and sense of injustice
- Feeling of loss of control
- Grief

The process of coping with the disease begins with the diagnosis. Coping is defined as:

> Attempting to reduce, compensate for, and process existing or expected stress caused by the disease through internal psychological (emotional and cognitive) processes or by targeted behavior and actions. This is a process-oriented event that depends on the various stages of the disease.

Coping strategies can be observed on the behavioral, cognitive, and emotional level. They include:

- Trust in doctors
- Fighting spirit: “I won't give up, I’ll keep fighting for my life and my health.”
- Denial: “I don’t have cancer; this is a wrong diagnosis; I know for sure that I just bumped myself.”

- Self-encouragement: every day, I consciously tell myself in the mirror: “I will make it.”
- Accept the disease as fate or an opportunity
- Distraction by reading, walking, telephone calls, visits, exercise, crafts, meditation, relaxation, etc.
- Creative expression (music, painting, etc.)
- Looking for information (internet!)
- Talking about distress, looking for help
- And much more

Coping with the disease is considered adaptive if:

- Adaptive tasks are completed successfully, for example accepting reality, orientation and focusing on the present, reduction of patterns of thinking that focus on fear of the future
- Absence of mental disorders
- Low negative and high positive affect
- Adequate (condition-appropriate) level of functioning (e.g., coping with everyday life)
- Satisfaction with and well-being in various areas of life
- Health-related quality of life

Mental Comorbidities

In 30 to 40 % of patients, phases with severe psychosocial distress occur during the course of the disease, and a mental disorder develops as part of a maladaptive coping process. Depending on the stage of the disease, 25 to 45 % of patients are expected to want or need professional psychological support. A baseline diagnostic assessment of psychological distress is therefore a continuous process and a quality characteristic of a patient-centered approach in oncology. A significant number of patients develop a mental disorder beyond an emotional reaction to the disease that can be considered normal. The cancer diagnosis may exacerbate a mental disorder that is pre-existing or in remission.

- Depressive symptoms: 15 to 50 %, in terminal stage 9 to 77 %
- Anxiety disorder: 10 to 50 %, in terminal stage 50 to 80 %
- Adjustment disorder: 5 to 50 %
- Post-traumatic stress disorder: 5 to 10 %
- Somatic symptom disorder: 9 to 14 %
- Sexual dysfunction: 25 to 50 %

Factors Influencing Coping with the Disease

Several factors influence the process of coping: The patient's health belief model, i.e., her assumptions about the cause of the cancer illness, her assumption of the efficiency of the medical treatment, and her own coping resources, is the central factor.

It is determined by disease-specific, interpersonal (e.g., personality traits such as resilience (resistance to stress)), and sociocultural factors (e.g., the meaning of cancer in the respective culture of the patient). The figure below illustrates the various factors that influence coping.

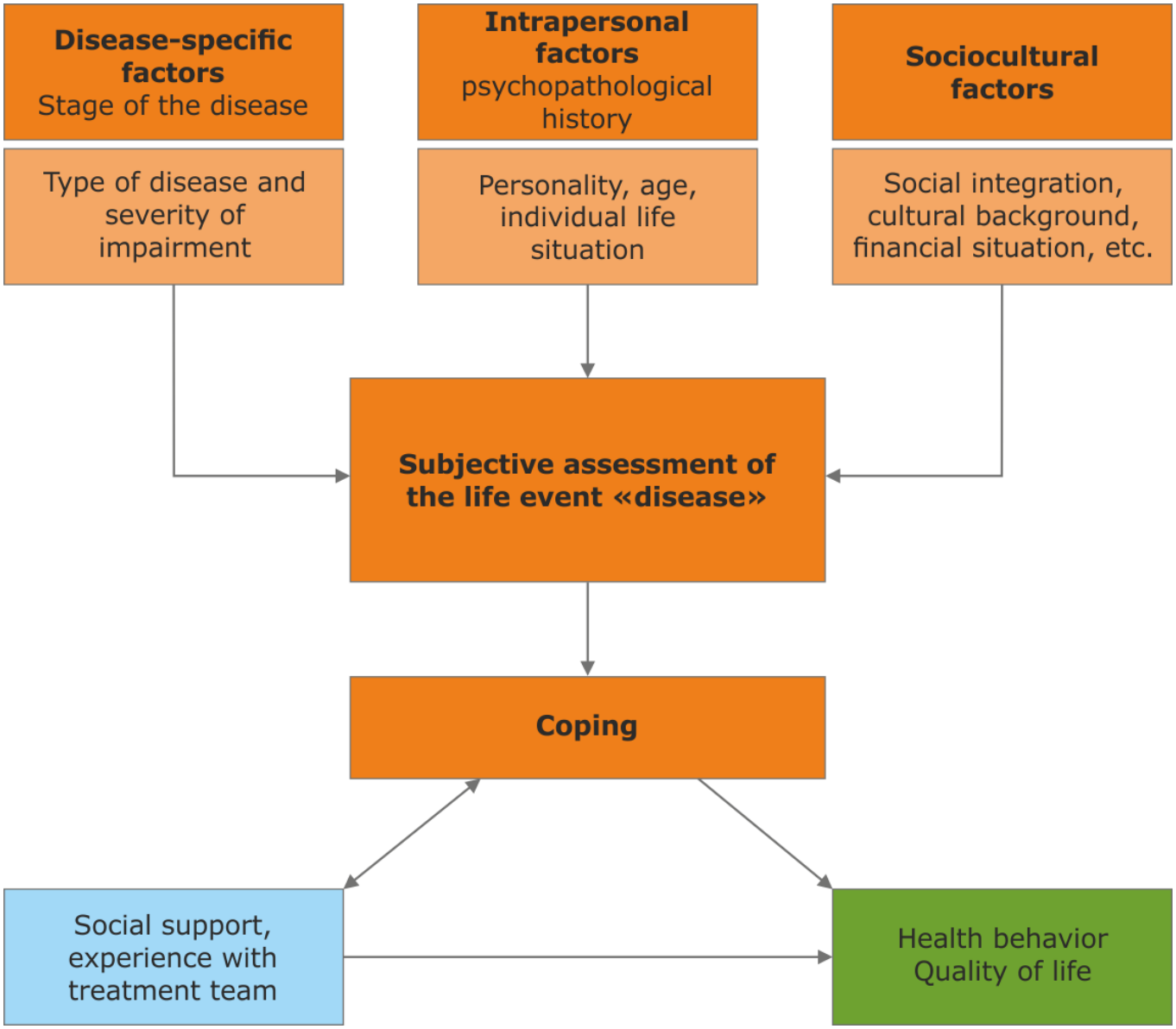


Figure 1

Psycho-Oncological approach as part of medical care

Diagnostic Phase

Frequent topics of patients

- Usually, high distress and great insecurity when waiting for the medical findings
- In case of diagnosis: acute fear of dying from the disease
- Fear of upcoming therapies and pain
- Insecurity about how to communicate with and inform the social environment (e.g., children)
- Strong need for information, many questions

Approach of the doctor

Table 1: Communicative skills

Breaking bad news	See script G17A "Communication and advice in gynecological consultations"
Information transfer, from suspicion to confirmation of the diagnosis	See script G17A "Communication and advice in gynecological consultations" Content: • Sensitive patient education, providing information on the disease and treatment options • Reduction of preconceptions about the consequences of a cancer diagnosis (isolation, suffering, death) • Working with diagrams and illustrations Caution: • Information overload of patient in case of limited processing capacity caused by emotional arousal • Not giving enough information / misinformation

Table 2: Identify and document

Social support	Evaluate patient's embedding in her social network and use of these resources. • "Are you close to people who can support you in this situation?" • "Who is it? How does this help you?" • "How well can you accept their help?" Support experienced by hospital/doctor: • "Is our care that you've experienced good so far?" • "Do you have any questions or requests for us as your treatment team?"
General resources	Identify resources that can be used for coping. • "This is a very stressful situation for many of my patients, and you will face some difficult decisions and changes. What would help you during the coming weeks?" • "How did you react to difficult life events in the past; what helped you?" • "What helps you relieve pressure?" • "When did you gain more distance to your situation in the past 48 hours, and what were you doing?"
Distress level	Identify disease-specific distress (see G17A) • Identify distress in patient's life "Each of my patients comes from a different background when she is diagnosed with cancer, i.e., sometimes patients also have other stressful problems. I would like to know more about your situation: • "Are there currently any other stressors in your life, e.g., excessive stress at work, conflicts, fears?" • "How long has this been going on?" • "How much does it affect you?" • "Did you get help and support for it?"
Previous experience with / knowledge about cancer	Most patients personally know someone who was diagnosed with cancer or have prior knowledge about the disease. This knowledge can influence the patient's health belief model. • "Do you know anyone else who suffered from cancer?" • "What do you know about cancer?"

Clarify need for support	• Home health care, Red Cross family support • Social workers • Psycho-oncological support individually, with a partner, with family members • Self-help groups / contact with affected patients Caution: • Do not impose help or “pathologize” patients.

Treatment Phase

Frequent topics of patients

- Dealing with side effects (see 2.2)
- Feelings of helplessness and loss of control
- Role change, no longer on equal emotional level with family and friends
- Feeling sick after previously feeling healthy
- Limited autonomy
- Time demands due to several doctor/hospital appointments
- Contact with many different, changing treatment providers
- Hope and confidence, but also fear that treatment will not work

Approach of the doctor

Table 3: Communicative skills

Information transfer	See script G17A "Communication and advice in gynecological consultations" Content: • Effect, course and side effects of different treatments • Distinguish short-, medium-term, and long-term side effects • Convey models that help understand and categorize specific undesirable effects • Example: Explain the effects of estrogen in the body and on cancer growth and deduce effects of anti-hormonal therapy on the body and cancer • Include the patient in decision-making processes, consider pros and cons, and make suggestions • Work with diagrams and drawings

Coping-oriented counseling	See script G17A "Communication and advice in gynecological consultations"

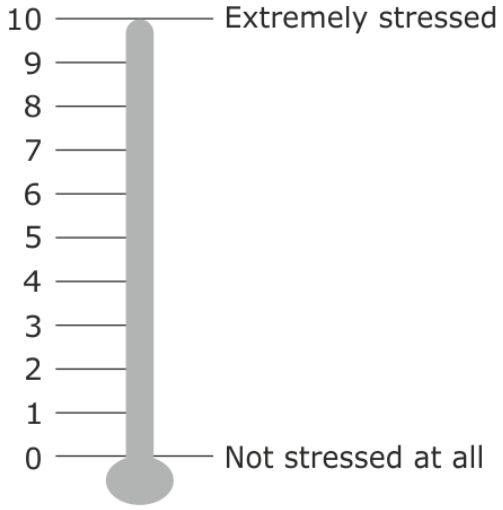
Table 4: Identify and document

Social support	see above “diagnostic phase”
General resources	see above “diagnostic phase”
Stress level and need for psycho-oncological support	1. Identify distress using distress thermometer (see below): • At the beginning of the treatment phase • At the end of chemotherapy and/or radiation • 1 year after diagnosis • If assessment > 5: Sensitivity of .93 for anxiety and/or .82 for depression, Specificity of .68 for anxiety and/or .62 for depression 2. Discuss: • “You indicated that your distress level is in the middle / the upper half on the thermometer. You weren’t feeling well at all this past week…” (wait) • “And emotionally, this shows in that you…” (mention checked emotional problems) 3. Ask about support: • “Do you feel that there is someone who supports you well in your current situation?” • Yes --> “Do you think that this is sufficient for you at the moment?“ Yes --> “I’m glad to hear that. If things change or if you would like to talk with a person trained in psycho-oncology, I can arrange that for you.” • No --> “Why don’t you consider meeting my colleague XY for a talk to look at your overall situation together and see how we can support you even better?”

Distress thermometer

1

On the thermometer on the right, circle the number (0-10) that best describes how stressed you have been feeling in the last week, including today.



2

Please indicate if you have had problems in one of the following areas in the last week, including today. Check **YES** or **NO** for every area.

YES	NO	Practical problems
○	○	Living situation
○	○	Insurance
○	○	Work / school
○	○	Transport
○	○	Childcare

YES	NO	Family problems
○	○	Dealing with your partner
○	○	Dealing with your children

YES	NO	Emotional problems
○	○	Worry
○	○	Fears
○	○	Sadness
○	○	Depression
○	○	Nervousness
○	○	Loss of interest in everday activities

YES	NO	Spiritual / religious issues
○	○	Concerning God
○	○	Loss of faith

YES	NO	Physical problems
○	○	Pain
○	○	Nausea
○	○	Exhaustion
○	○	Sleep
○	○	Moving / mobility
○	○	Washing / Dressing
○	○	Appearance
○	○	Breathing
○	○	Inflammation in the mouth
○	○	Eating / nutrition
○	○	Indigestion
○	○	Constipation
○	○	Diarrhea
○	○	Changes in urination
○	○	Fever
○	○	Dry / itchy skin
○	○	Dry / stuffy nose
○	○	Tingling in hand / feet
○	○	Feeling swollen / bloated
○	○	Memory / concentration
○	○	Sexual problems

Other problems _____

3

Please indicate if you would like to counseling with a psycho-oncologist. **YES** ○ **NO** ○

Figur 2

Follow-up

Frequent topics of patients

- Getting back to normalcy
- Finding meaning in the disease
- Complementary therapies (e.g., homeopathy, acupuncture, aromatherapy, phytotherapy, relaxation techniques, kinesitherapy,..)
- Looking for ways to contribute to maintaining one's health (yoga, nutrition, exercise, etc.)
- Identity: cancer patient or healthy ("survivor")?
- Coping continues for the patient, whereas for her environment, the disease already belongs to the past
- Great fear and insecurity prior to follow-up appointments and examinations
- Heightened physical self-awareness and refined perception --> interpret body signals as possible signs of relapse
- Side effects (see 2.2)
- Changes in sexuality, body image, perceived femininity, and confidence in the body
- Fatigue or/and pain

Approach of the doctor

Table 5: Communicative skills

Information transfer	See script G17A "Communication and advice in gynecological consultations" Content: see treatment phase Promote patient health literacy
Coping-oriented counseling	See script G17A "Communication and advice in gynecological consultations"

Table 6: Identify and document

Sexuality	Address actively, because patients are often insecure Treat lack of lubrication to prevent developing a pain cycle Evaluate/discuss indication for sex counseling

General climacteric symptoms	Especially in patients that are premenopausal at first diagnosis
Distress level and need for psycho-oncological support	Use the distress thermometer 1 year after first diagnosis to identify delayed coping with the disease (see above "treatment phase")

Relapse/Progression

Frequent topics of patients

- Fear of a shortened life span, losing hope for healing
- Question: “How much longer?”
- Difficulty with medical distinction between curative and palliative care (The patient knows that the disease is no longer curable. Now the goal is to ensure the best possible quality of life and facilitate living with the disease. This is palliative medicine. Curative medicine aims at healing).
- Fear of pain and death
- Exacerbated fear of isolation and loss
- Increased dependency on doctor
- Search for miracle cure
- Spirituality
- Ambivalence between fully utilizing all possible therapeutic options and maintaining a high quality of life
- Disappointment and feeling of failure of treatments to date and of own efforts (anger, outrage against medical team or oneself)

Approach of the doctor

Table 7: Communicative skills

General aspects	• Communication is put to the test in case of progression because the patient might be more mentally vulnerable and skeptical • Empathetic and active listening, mirroring, open communication with sufficient time and in a good setting • Include family members • Convey hope • Give room to existential topics

Emotion-oriented communication	Possible questions • “I wonder what that means for you…” • “I would like to understand you better. Is it correct that...?”

Table 8: Identify and document

Distress level and need for psycho-oncological support	see above “treatment phase”

Terminal Stage

Frequent topics of patients

- Symptoms (weakness, neuropsychiatric symptoms, pain, respiratory symptoms, insomnia, etc.)
- Relationship conflicts
- Parting and loss (sadness with respect to loss)
- Isolation
- Worries about family members
- Desire to control/plan the last stage of life, or persistent denial
- Fear of and coping with pain
- Question: “How much longer?”
- Spirituality

Approach of the doctor

Table 9: Communicative skills

Tasks	• Supporting, holding • Sympathy, dignity, and integrity • Time for counseling and regular counseling • Include and inform family members • Ask about unfinished tasks and topics (e.g., parting from people, clarification/reconciliation, last will, etc.) • Discuss living will

Discussions with family in case of death	• Deliver news with empathy at the right place, while taking enough time • Offer additional consultations • Discuss and carry out rituals • Answer questions of children honestly and openly

Table 10: Identify and document

Spirituality	Possible question: • What do you think about it?
Dying phase	• Establish a chain of care together with the family • Wishes concerning medical treatment (e.g., no reanimation, no resuscitation, no life-prolonging measures) • Discuss rituals

Psycho-oncological interventions

Goal of Psycho-Oncological Interventions

Improving quality of life

- Reduction of disease- or treatment-related symptoms (such as nausea, vomiting, pain, insomnia, etc.)
- Reduction of trauma reactions, fear, depression, hopelessness, and helplessness
- Strengthen potential for self-help (self-control, personal responsibility, coping: Coping refers to the way the patient deals with and/or manages the disease. Does she have a fighting, optimistic, hopeful attitude, or feelings of helplessness and hopelessness; can she accept things she cannot change and focus on things she can change, etc.)
- Improve self-confidence and attitude towards cancer
- Provide help to clarify misunderstandings and misinformation
- Provide support in dealing with social environment and improving social support
- Improve doctor-patient relationship, support communication between patient, partner, family, and medical personnel
- Support remaining health

Active Components of Psycho-Oncological Interventions

- Structure: Structured psychoeducational programs are more effective than unstructured discussion groups.
- Format: Group interventions are more effective than individual therapy.
- Focus: Interventions promoting self-efficacy are more effective.
- Duration: Longer interventions are more effective than shorter ones.

Psychopharmacological Interventions

Because various organ systems are involved, the liver and kidney metabolism are affected due to undesirable effects and complications, and pharmacological interactions are unpredictable, psychotropic drugs are often only prescribed reluctantly, potentially causing an undersupply.
Antidepressants, especially SSRIs, and sedatives are employed to manage anxiety and stress during the acute shock phase, prior to stressful interventions, in palliative situations, and in the case of delirium.

The following factors increase the risk of drug interactions

- High protein binding
- Cytochrome P450-dependent metabolism
- Narrow therapeutic index

Table 11: Examples for particularly suitable antidepressants in oncology

Active ingredient Products	Desired effect	Dosage in oncology (mg)	Side effects
Mirtazapine Remeron®, Mirtazapine Spirig®	Sedating, sleep-inducing	15-30 per day	Weight gain, Tiredness
Citalopram Seropram®, Ccitalopram Sandoz®	Activating, non-sedating, Anxiolytic	20-40 per day	Gastrointestinal side effects, Confusion and initial agitation
Escitalopram Cipralex®	Activating, non-sedating, Anxiolytic	10-20 per day	Gastrointestinal side effects, Confusion and initial agitation
Venlafaxine, Efexor®, Venlax®	Activating, non-sedating, Anxiolytic	75-150 per day	Hypertension, Tachycardia, Headaches

Psycho-Oncological Treatment Options

During surgical and oncological treatment and especially during the diagnostic and treatment phase as well as follow-up, approx. 30-50% of patients have an indication for supplementary psycho-oncological treatment by appropriately trained psychologists and psychiatrists. Duration, intensity and type of treatment differ depending on the problem and phase of the disease, ranging from a single crisis intervention to multi-year support.

Interventions are varied and include, among others:

- Clarifying, supportive, and solution-oriented counseling in individual, couple, family, and group settings
- Information on relaxation techniques (e.g., relaxation, breathing) and

mindfulness training (e.g., yoga, meditation)
- Development of concrete coping strategies (e.g., for anxiety, pain, and depression)
- Sexual therapy interventions
- Bibliotherapy
- Painting and art therapy
- Exercise and movement therapy (e.g., kinesiotherapy or/and kinesiology)
- Body-based methods (autogenic training)
- Etc.

Co-therapy by a psycho-oncologist should be offered to every cancer patient at the beginning and during the course of treatment (during treatment and at the beginning of follow-up care, in case of relapse, and during the palliative phase).

Treatment Options

Treatment providers and affected patients can find experts, treatment options, self-help groups, information and brochures at www.krebsliga.ch (search for option and canton is possible). Various brochures and information for Germany are available at www.krebshilfe.de.

Krebsliga Switzerland

Krebshilfe Germany

Further Training

In Switzerland, further training in psycho-oncology is offered as two-year advanced training (www.psychoonkologie.ch/weiterbildung-psychoonkologie.html).

Information on further training in Germany is available at http://www.wpo-ev.de/.

Bibliography

Nr.	Title	Author	Journal	Volume	Pages	Year
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[P02]	Effects of psychosocial interventions on quality of life in adult cancer patients: meta-analysis of 37 published controlled outcome studies	Rehse B, Pukrop R	Patient education and counseling	50(2)	179-186	2003
[P03]	Long-term quality of life in gynecological cancer survivors	Gonçalves V	Curr Opin Obstet Gynecol	22(1)	30-35	2010
[P04]	A systematic review of sexual concerns reported by gynecological cancer survivors	Abbott-Anderson K, Kwekkeboom KL	Gynecol Oncol	124(3)	477-489	2012

Nr.	Title	Author	Journal	Volume	Pages	Year
[P05]	Are we there yet? The state of the evidence base for guidelines on breaking bad news to cancer patients	Paul CL, Clinton-McHarg T, Sanson-Fisher RW, Douglas H, Webb G	Eur J Cancer	45(17)	2960-2966	2009
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